

Vecuronium: Drug information - UpToDate

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For additional information see "Vecuronium: Patient drug information" and "Vecuronium: Pediatric drug information"

For abbreviations, symbols, and age group definitions show table

ALERT: US Boxed Warning

Experienced personnel:

This drug should be administered by adequately trained individuals familiar with its actions, characteristics, and hazards.

Pharmacologic Category

- Neuromuscular Blocker Agent, Nondepolarizing

Dosing: Adult

Dosage guidance:

Dosing: Doses must be individualized due to interpatient variability, patient-specific factors, and/or comorbidities (eg, severe trauma, burn injury) (Ref).

Endotracheal intubation

Endotracheal intubation:

Note: Ensure adequate pain control and sedation prior to and during administration of neuromuscular blockade (Ref).

Endotracheal intubation:

IV: 0.1 to 0.2 mg/kg once (Ref).

Rapid-sequence intubation, neuromuscular blockade (alternative agent) (off-label use):

Note: Only consider use when succinylcholine or rocuronium are contraindicated or not available; may have a delayed onset of action when compared to succinylcholine or rocuronium (Ref).

IV: 0.1 to 0.2 mg/kg once (Ref).

Mechanically ventilated patients in the ICU, neuromuscular blockade

Mechanically ventilated patients in the ICU, neuromuscular blockade: Note: Ensure adequate pain control and sedation prior to and during administration of neuromuscular blockade. May use to facilitate mechanical ventilation (eg, refractory moderate to severe acute respiratory distress

syndrome; refractory, life-threatening status asthmaticus; or shivering from therapeutic hypothermia) (Ref). Refer to institutional protocols. Titration specifics may vary; a titration example is provided below.

Intermittent dose: IV: 0.1 to 0.2 **mg/kg**; may repeat based on clinical response and neuromuscular monitoring (eg, train-of-four) (Ref); for treatment of shivering from therapeutic hypothermia, may use 8 to 12 **mg**; repeat as needed to maintain adequate control (Ref).

Continuous infusion: IV: 0.08 to 0.1 **mg/kg** initial loading dose, followed by a continuous infusion of 0.8 to 1.7 **mcg/kg/minute**; titrate in increments of 0.3 **mcg/kg/minute** or by 50% reductions of previous dose based on clinical response and neuromuscular monitoring (eg, train-of-four). Usual dosage range: 0.8 to 1.2 **mcg/kg/minute** (Ref).

Neuromuscular blockade during general anesthesia

Neuromuscular blockade during general anesthesia (adjunctive therapy):

Maintenance dosing: Note: Inhaled anesthetic agents (eg, desflurane, isoflurane, sevoflurane) prolong the duration of action of vecuronium and may potentiate the neuromuscular blockade.

Intermittent dose: IV: 0.01 to 0.015 **mg/kg**; may repeat as needed based on clinical response and neuromuscular monitoring (eg, train-of-four) (Ref).

Continuous infusion: IV: 0.8 to 1.7 **mcg/kg/minute** as an initial continuous infusion; titrate as needed based on clinical response and neuromuscular monitoring (eg, train-of-four) (Ref).

Dosage adjustment for concomitant therapy: Significant drug interactions exist, requiring dose/frequency adjustment or avoidance. Consult drug interactions database for more information.

Dosing: Kidney Impairment: Adult

There are no dosage adjustments provided in the manufacturer's labeling. However, need for dosage adjustment unlikely as limited excretion in the urine (Ref).

Dosing: Liver Impairment: Adult

There are no dosage adjustments provided in the manufacturer's labeling. However, dosage reduction may be necessary in patients with liver disease.

Dosing: Obesity: Adult

The recommendations for dosing in patients with obesity are based upon the best available evidence and clinical expertise. Senior Editorial Team: Jeffrey F. Barletta, PharmD, FCCM; Manjunath P. Pai, PharmD, FCP; Jason A. Roberts, PhD, BPharm (Hons), B App Sc, FSHP, FISAC.

Class 1, 2, or 3 obesity (BMI ≥ 30 kg/m²):

IV: Use **ideal body weight** for weight-based dosing calculations, then titrate to clinical effect. In patients with extreme obesity (eg, BMI ≥ 50 kg/m²), or when underdosing is a concern, consider using **adjusted body weight** for weight-based dosing calculations, then titrate to clinical effect; however, duration of action may be prolonged. Clinicians should **not** change dosing weight from one weight metric to another during therapy (ie, actual body weight to/from adjusted body weight or ideal body weight) (Ref). Refer to adult dosing for indication-specific doses.

Rationale for recommendations:

Nondepolarizing neuromuscular blocking agents (NMBA) are hydrophilic compounds with a small V_d ; thus, distribution of vecuronium into adipose tissue is limited (Ref). There are no studies evaluating the most appropriate weight metric in patients with obesity receiving sustained continuous infusions. Data with NMBAs in patients with obesity predominantly originate from studies evaluating either single bolus or incremental doses in patients undergoing surgical procedures. Studies where vecuronium was dosed using actual body weight have demonstrated prolonged recovery times in patients with obesity compared to patients who are normal weight or overweight, suggesting alternative weight-based metrics (eg, ideal body weight, lean body weight) may be preferred (Ref).

Dosing: Older Adult

Refer to adult dosing. Dose selection should be cautious, at low end of dosage range, and titration should be slower to evaluate response.

Dosing: Pediatric

(For additional information see "Vecuronium: Pediatric drug information")

Note: Dose to effect; doses will vary due to interpatient variability. Dosing in obese patients should be calculated using ideal body weight (Ref).

Neuromuscular blockade

Neuromuscular blockade:

Infants:

IV: 0.08 to 0.1 mg/kg/dose, repeat as needed (Ref); **Note:** If intubation is performed using succinylcholine, the initial dose of vecuronium may be reduced to 0.04 to 0.06 mg/kg with inhalation anesthesia and 0.05 to 0.06 mg/kg with balanced anesthesia (Ref).

Continuous IV infusion: 0.8 to 1.7 **mcg**/kg/minute (0.05 to 0.1 mg/kg/**hour**) (Ref).

Children and Adolescents:

IV: 0.08 to 0.1 mg/kg/dose, repeat as needed (Ref); **Note:** If intubation is performed using succinylcholine, the initial dose of vecuronium may be reduced to 0.04 to 0.06 mg/kg with inhalation anesthesia and 0.05 to 0.06 mg/kg with balanced anesthesia. Children 1 to 10 years may require slightly higher initial doses and more frequent supplementation (Ref).

Continuous IV infusion: 0.8 to 2.5 **mcg**/kg/minute (0.05 to 0.15 mg/kg/**hour**) (Ref).

Rapid sequence intubation

Rapid sequence intubation: Limited data available:

Infants, Children, and Adolescents: IV, Intraosseous: 0.1 to 0.3 mg/kg (Ref).

Dosage adjustment for concomitant therapy: Significant drug interactions exist, requiring dose/frequency adjustment or avoidance. Consult drug interactions database for more information.

Dosing: Kidney Impairment: Pediatric

There are no dosage adjustments provided in manufacturer's labeling. Although patients with kidney impairment do not experience clinically significant prolongation of neuromuscular blockade with vecuronium, patients who are anephric may have a prolonged clinical duration; as a result, a lower initial dose should be considered.

Dosing: Liver Impairment: Pediatric

There are no dosage adjustments provided in manufacturer's labeling; however, dose reductions may be necessary in patients with liver disease.

Adverse Reactions

The following adverse drug reactions and incidences are derived from product labeling unless otherwise specified.

<1%, postmarketing, and/or case reports: Bradycardia, circulatory shock, edema, flushing, hypersensitivity reaction (including erythema, hypotension, tachycardia, urticaria), pruritus, skin rash

Contraindications

Hypersensitivity to vecuronium or any component of the formulation

Warnings/Precautions

Concerns related to adverse effects:

- Anaphylaxis: Severe anaphylactic reactions have been reported with vecuronium use; some life-threatening and fatal. Appropriate emergency treatment (including epinephrine 1 mg/mL) should be

immediately available during use. Use caution in patients with previous anaphylactic reactions to other neuromuscular-blocking agents.

- Prolonged paralysis: Some patients may experience delayed recovery of neuromuscular function after administration (especially after prolonged use). Other factors associated with delayed recovery should be considered (eg, corticosteroid use, disease-related conditions).

Disease-related concerns:

- Burn injury: Resistance may occur in burn patients ($\geq 20\%$ of total body surface area), usually several days after the injury, and may persist for several months after wound healing (Han 2009).
- Conditions that may *antagonize* neuromuscular blockade (decreased paralysis): Respiratory alkalosis, hypercalcemia, demyelinating lesions, peripheral neuropathies, denervation, and muscle trauma may result in antagonism of neuromuscular blockade (ACCM/SCCM/ASHP [Murray 2002]; Greenberg 2013; Miller 2010; Naguib 2002).
- Conditions that may *potentiate* neuromuscular blockade (increased paralysis): Electrolyte abnormalities (eg, severe hypocalcemia, severe hypokalemia, hypermagnesemia), neuromuscular diseases, metabolic acidosis, respiratory acidosis, Eaton-Lambert syndrome, and myasthenia gravis may result in potentiation of neuromuscular blockade (Greenberg 2013; Miller 2010; Naguib 2002).
- Hepatic impairment: Use with caution in patients with hepatic impairment; clinical duration may be prolonged.
- Renal impairment: In general, patients with renal impairment do not experience clinically significant prolongation of neuromuscular blockade with vecuronium; however, in patients who are anephric, the clinical duration may be prolonged.
- Respiratory disease: Use with caution in patients with underlying respiratory disease.

Special populations:

- Older adult: Use with caution in the elderly, effects and duration are more variable; dosage reduction may be considered.
- Immobilized patients: Resistance may occur in patients who are immobilized.
- Pediatric: Children 1-10 years of age may require slightly higher initial doses and slightly more frequent supplementation.

Dosage form specific issues:

- Benzyl alcohol and derivatives: Diluent (bacteriostatic water for injection) contains benzyl alcohol; large amounts of benzyl alcohol (≥ 99 mg/kg/day) have been associated with a potentially fatal toxicity ("gasping syndrome") in neonates; the "gasping syndrome" consists of metabolic acidosis, respiratory distress, gasping respirations, CNS dysfunction (including convulsions, intracranial hemorrhage), hypotension, and cardiovascular collapse (AAP ["Inactive" 1997]; CDC 1982); some data suggests that benzoate displaces bilirubin from protein binding sites (Ahlfors 2001); avoid or use dosage forms containing benzyl alcohol with caution in neonates. See manufacturer's labeling.

Other warnings/precautions:

- Appropriate use: Maintenance of an adequate airway and respiratory support is critical. All patients should receive eye care including liberal use of lubricating drops, gel, or ointment and eyelids should remain closed during continuous neuromuscular blockade to protect against damage to the cornea (ulceration and drying).
- Experienced personnel: **[US Boxed Warning]: Should be administered by adequately trained individuals familiar with its use.**
- Risk of medication errors: Accidental administration may be fatal. Confirm proper selection of intended product, store vial so the cap and ferrule are intact and the possibility of selecting the wrong product is minimized, and ensure that the intended dose is clearly labeled and communicated, when applicable.

Dosage Forms: US

Excipient information presented when available (limited, particularly for generics); consult specific product labeling.

Solution Reconstituted, Intravenous, as bromide:

Generic: 10 mg (1 ea); 20 mg (1 ea)

Solution Reconstituted, Intravenous, as bromide [preservative free]:

Generic: 10 mg (1 ea); 20 mg (1 ea)

Generic Equivalent Available: US

Yes

Pricing: US

Solution (reconstituted) (Vecuronium Bromide Intravenous)

10 mg (per each): \$4.74 - \$10.20

20 mg (per each): \$9.60 - \$20.40

Disclaimer: A representative AWP (Average Wholesale Price) price or price range is provided as reference price only. A range is provided when more than one manufacturer's AWP price is available and uses the low and high price reported by the manufacturers to determine the range. The pricing data should be used for benchmarking purposes only, and as such should not be used alone to set or adjudicate any prices for reimbursement or purchasing functions or considered to be an exact price for a single product and/or manufacturer. Medi-Span expressly disclaims all warranties of any kind or nature, whether express or implied, and assumes no liability with respect to accuracy of price or price range data published in its solutions. In no event shall Medi-Span be liable for special, indirect, incidental, or consequential damages arising from use of price or price range data. Pricing data is updated monthly.

Administration: Adult

IV: For IV administration only; do not administer IM. Concentration of 1 mg/mL may be administered by rapid IV injection; may also be used for IV infusion in fluid-restricted patients.

Preparation for Administration: Adult

Reconstitute with compatible solution (eg, D5W, NS) for injection to final concentration of 1 mg/mL. May further dilute reconstituted vial to 0.1 to 0.2 mg/mL in a compatible solution for IV infusion.

Usual Infusion Concentrations: Adult

IV infusion: 10 mg in 100 mL (concentration: 0.1 mg/mL), 20 mg in 100 mL (concentration: 0.2 mg/mL), **or** 50 mg in 50 mL (concentration: 1 mg/mL) of D₅W or NS

Administration: Pediatric

Parenteral:

IV bolus: Administer undiluted (reconstituted solution at 1 mg/mL) by rapid direct injection.

Continuous IV infusion: Administer via an infusion pump.

Preparation for Administration: Pediatric

Parenteral:

IV bolus: Reconstitute with provided diluent (contains benzyl alcohol) or compatible solution (eg, SWFI, NS) to a final concentration of 1 mg/mL; do NOT reconstitute with provided diluent for neonatal use.

Continuous IV infusion: Further dilute reconstituted vial in a compatible solution (eg, D5W, NS) for IV infusion; the manufacturer suggests a final concentration range of 0.1 to 0.2 mg/mL; however,

concentrations as high as 1 mg/mL have been reported (Ref). ASHP recommends a standard concentration of 1 mg/mL (Ref).

Usual Infusion Concentrations: Pediatric

IV infusion: 0.1 mg/mL, 0.2 mg/mL, 1 mg/mL

Storage/Stability

Store intact vials at 20°C to 25°C (68°F to 77°F). Vials reconstituted with bacteriostatic water for injection (BWFI) may be stored for 5 days under refrigeration or at room temperature. Vials reconstituted with other compatible diluents (including D5W, D5NS, LR, NS) should be stored under refrigeration and used within 24 hours. According to the 2020 to 2021 *ISMP Targeted Medication Safety Best Practices for Hospitals*, neuromuscular blockers should be segregated, sequestered, and differentiated from all other medication wherever they are stored.

Use: Labeled Indications

Endotracheal intubation: To facilitate endotracheal intubation.

Mechanically ventilated patients in the ICU, neuromuscular blockade: Provide skeletal muscle relaxation during mechanical ventilation in ICU patients.

Neuromuscular blockade during general anesthesia: As an adjunct to general anesthesia to provide skeletal muscle relaxation during surgery.

Note: Neuromuscular blockade does not provide pain control, sedation, or amnestic effects. Ensure adequate pain control and sedation prior to and during administration of neuromuscular blockade (SCCM [Murray 2016]).

Use: Off-Label: Adult

Rapid-sequence intubation, neuromuscular blockade

Medication Safety Issues

Sound-alike/look-alike issues:

Vecuronium may be confused with valproate sodium, vancomycin

Norcuron may be confused with Narcan

High alert medication:

The Institute for Safe Medication Practices (ISMP) includes this medication among its list of drug classes (neuromuscular blocker agent) which have a heightened risk of causing significant patient harm when used in error (ISMP List of High-Alert Medications in Acute Care Settings ©ISMP 2024).

Other safety concerns:

According to the 2020 to 2021 ISMP targeted medication safety best practices for hospitals, neuromuscular blockers should be segregated, sequestered, and differentiated from all other medication wherever they are stored. This includes:

Only storing in places within the hospital that they are routinely used.

Placing in sealed boxes or in rapid-sequence intubation kits (preferred).

Limiting availability in automated dispensing cabinets to perioperative, labor and delivery, critical care, and emergency departments only.

Placing in separate lidded containers within the pharmacy refrigerator or other isolated pharmacy storage area.

Affixing an auxiliary label to clearly communicate respiratory paralysis will occur and ventilation required on all storage bins and/or automated dispensing pockets/drawers (exception anesthesia-prepared syringes) stating one of the following:

Warning: Causes Respiratory Arrest - Patient Must Be Ventilated.

Warning: Paralyzing Agent - Causes Respiratory Arrest.

Warning: Causes Respiratory Paralysis - Patient Must Be Ventilated.

Metabolism/Transport Effects

None known.

Drug Interactions

(For additional information: Launch drug interactions program)

Note: Interacting drugs may **not be individually listed below** if they are part of a group interaction (eg, individual drugs within “CYP3A4 Inducers [Strong]” are NOT listed). For a complete list of drug interactions by individual drug name and detailed management recommendations, use the drug interactions program by clicking on the “Launch drug interactions program” link above.

Acetylcholinesterase Inhibitors: May decrease neuromuscular-blocking effects of Neuromuscular-Blocking Agents (Nondepolarizing). *Risk C: Monitor*

Aminoglycosides: May increase therapeutic effects of Neuromuscular-Blocking Agents. *Risk C: Monitor*

Bacitracin (Systemic): May increase neuromuscular-blocking effects of Neuromuscular-Blocking Agents. *Risk C: Monitor*

Botulinum Toxin-Containing Products: May increase neuromuscular-blocking effects of Neuromuscular-Blocking Agents. *Risk C: Monitor*

Bromperidol: May increase neuromuscular-blocking effects of Neuromuscular-Blocking Agents. *Risk C: Monitor*

Calcium Channel Blockers: May increase neuromuscular-blocking effects of Neuromuscular-Blocking Agents (Nondepolarizing). *Risk C: Monitor*

Capreomycin: May increase neuromuscular-blocking effects of Neuromuscular-Blocking Agents. *Risk C: Monitor*

CarBAMazepine: May decrease serum concentrations of Vecuronium. *Risk C: Monitor*

Cardiac Glycosides: Neuromuscular-Blocking Agents may increase arrhythmogenic effects of Cardiac Glycosides. *Risk C: Monitor*

Clindamycin (Topical): May increase neuromuscular-blocking effects of Neuromuscular-Blocking Agents. *Risk C: Monitor*

Colistimethate: May increase neuromuscular-blocking effects of Neuromuscular-Blocking Agents. Management: If possible, avoid concomitant use of these products. Monitor for deeper, prolonged neuromuscular-blocking effects (respiratory paralysis) in patients receiving concomitant neuromuscular-blocking agents and colistimethate. *Risk D: Consider Therapy Modification*

Corticosteroids (Systemic): Neuromuscular-Blocking Agents (Nondepolarizing) may increase adverse neuromuscular effects of Corticosteroids (Systemic). Increased muscle weakness, possibly progressing to polyneuropathies and myopathies, may occur. Management: If concomitant therapy is required, use the lowest dose for the shortest duration to limit the risk of myopathy or neuropathy. Monitor for new onset or worsening muscle weakness, reduction or loss of deep tendon reflexes, and peripheral sensory decrements *Risk D: Consider Therapy Modification*

CycloSPORINE (Systemic): May increase neuromuscular-blocking effects of Neuromuscular-Blocking Agents. *Risk C: Monitor*

Dantrolene: May increase neuromuscular-blocking effects of Vecuronium. *Risk C: Monitor*

Esketamine (Injection): May increase neuromuscular-blocking effects of Neuromuscular-Blocking Agents. *Risk C: Monitor*

Fosphenytoin-Phenytoin: May decrease neuromuscular-blocking effects of Neuromuscular-Blocking Agents (Nondepolarizing). Specifically, this effect is observed with chronic phenytoin administration. Fosphenytoin-Phenytoin may increase neuromuscular-blocking effects of Neuromuscular-Blocking Agents (Nondepolarizing). Specifically, this effects is observed with acute/short term phenytoin administration. *Risk C: Monitor*

Gepotidacin: May decrease therapeutic effects of Neuromuscular-Blocking Agents (Nondepolarizing). *Risk C: Monitor*

Inhalational Anesthetics: May increase neuromuscular-blocking effects of Neuromuscular-Blocking Agents (Nondepolarizing). Management: When initiating a non-depolarizing neuromuscular blocking agent (NMBA) in a patient receiving an inhalational anesthetic, initial NMBA doses should be reduced 15% to 25% and doses of continuous infusions should be reduced 30% to 60%. *Risk D: Consider Therapy Modification*

Ketamine: May increase neuromuscular-blocking effects of Neuromuscular-Blocking Agents. *Risk C: Monitor*

Ketorolac (Nasal): May increase adverse/toxic effects of Neuromuscular-Blocking Agents (Nondepolarizing). Specifically, episodes of apnea have been reported in patients using this combination. *Risk C: Monitor*

Ketorolac (Systemic): May increase adverse/toxic effects of Neuromuscular-Blocking Agents (Nondepolarizing). Specifically, episodes of apnea have been reported in patients using this combination. *Risk C: Monitor*

Lincosamide Antibiotics: May increase neuromuscular-blocking effects of Neuromuscular-Blocking Agents. *Risk C: Monitor*

Lithium: May increase neuromuscular-blocking effects of Neuromuscular-Blocking Agents. *Risk C: Monitor*

Local Anesthetics: May increase neuromuscular-blocking effects of Neuromuscular-Blocking Agents. *Risk C: Monitor*

Loop Diuretics: May decrease neuromuscular-blocking effects of Neuromuscular-Blocking Agents. Loop Diuretics may increase neuromuscular-blocking effects of Neuromuscular-Blocking Agents. *Risk C: Monitor*

Magnesium Salts: May increase neuromuscular-blocking effects of Neuromuscular-Blocking Agents. *Risk C: Monitor*

MetroNIDAZOLE (Systemic): May increase neuromuscular-blocking effects of Vecuronium. *Risk C: Monitor*

Minocycline (Systemic): May increase neuromuscular-blocking effects of Neuromuscular-Blocking Agents. *Risk C: Monitor*

Ornidazole: May increase neuromuscular-blocking effects of Vecuronium. *Risk C: Monitor*

Pancuronium: May increase adverse/toxic effects of Neuromuscular-Blocking Agents. Pancuronium may increase therapeutic effects of Neuromuscular-Blocking Agents. *Risk C: Monitor*

Pholcodine: May increase adverse/toxic effects of Neuromuscular-Blocking Agents. Specifically, anaphylaxis has been reported. *Risk C: Monitor*

Piperacillin: May increase neuromuscular-blocking effects of Vecuronium. *Risk C: Monitor*

Polymyxin B: May increase neuromuscular-blocking effects of Neuromuscular-Blocking Agents. *Risk C: Monitor*

Procainamide: May increase neuromuscular-blocking effects of Neuromuscular-Blocking Agents. *Risk C: Monitor*

QuiNIDine: May increase neuromuscular-blocking effects of Neuromuscular-Blocking Agents. *Risk C: Monitor*

QuiNINE: May increase neuromuscular-blocking effects of Neuromuscular-Blocking Agents. *Risk X: Avoid*

Tetracyclines: May increase neuromuscular-blocking effects of Neuromuscular-Blocking Agents. *Risk C: Monitor*

Thiazide and Thiazide-Like Diuretics: May increase neuromuscular-blocking effects of Neuromuscular-Blocking Agents (Nondepolarizing). *Risk C: Monitor*

Vancomycin: May increase neuromuscular-blocking effects of Neuromuscular-Blocking Agents. *Risk C: Monitor*

Pregnancy Considerations

The pharmacokinetics of vecuronium are altered during pregnancy. Use in cesarean section has been reported; umbilical venous concentrations were 11% of maternal values at delivery.

Breastfeeding Considerations

It is not known if vecuronium is present in breast milk. The manufacturer recommends that caution be exercised when administering vecuronium to breastfeeding women.

Monitoring Parameters

Vital signs (heart rate, blood pressure, respiratory rate); degree of muscle paralysis (eg, presence of spontaneous movement, ventilator asynchrony, shivering; use a peripheral nerve stimulator with train of four monitoring along with clinical assessments)

In the ICU setting, prolonged paralysis and generalized myopathy, following discontinuation of agent, may be minimized by appropriately monitoring degree of blockade.

Mechanism of Action

Blocks acetylcholine from binding to receptors on motor endplate inhibiting depolarization

Pharmacokinetics (Adult Data Unless Noted)

Onset of action:

Good intubation conditions: Within 2 to 4 minutes (Engstrom 2023; Price 2012).

Maximum neuromuscular blockade: Within 3 to 5 minutes.

Duration: Under balanced anesthesia (time to recovery to 25% of control): 25 to 40 minutes; recovery 95% complete ~45 to 65 minutes after injection of intubating dose; hypothermia may prolong the duration of action.

Distribution: V_d : 0.3 to 0.4 L/kg.

Protein binding: 60% to 80%.

Metabolism: Active metabolite: 3-desacetyl vecuronium ($1/2$ the activity of parent drug).

Half-life, elimination:

Infants: 65 minutes.

Children: 41 minutes.

Healthy adult surgical patients and renal failure patients undergoing transplant surgery: 65 to 75 minutes; Late pregnancy: 35 to 40 minutes.

Half-life, distribution: Adults: 4 minutes.

Excretion: Primarily feces (35% to 50%); urine (15% to 50% as unchanged drug and metabolites) (Price 2012).

Patient Counseling Points

(For additional information see "Vecuronium: Patient drug information")

What is this drug used for?

- It is used to calm muscles during surgery.
- It is used to calm muscles while on a breathing machine.

WARNING/CAUTION: Even though it may be rare, some people may have very bad and sometimes deadly side effects when taking a drug. Tell your doctor or get medical help right away if you have any of the following signs or symptoms that may be related to a very bad side effect:

- Signs of an allergic reaction, like rash; hives; itching; red, swollen, blistered, or peeling skin with or without fever; wheezing; tightness in the chest or throat; trouble breathing, swallowing, or talking; unusual hoarseness; or swelling of the mouth, face, lips, tongue, or throat.

Note: This is not a comprehensive list of all side effects. Talk to your doctor if you have questions.

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Brand Names: International

International Brand Names by Country

For country code abbreviations (show table)

- (AE) United Arab Emirates: Norcuron;
- (AR) Argentina: Bromuro de vecuronio pharmavial | Gobbicuronio | Rivecrum | Vecuron | Vecuronio gray | Vecuronio northia | Vecuronio richet;
- (AT) Austria: Norcuron;
- (AU) Australia: Norcuron | Vecure | Vecuronium sun;
- (BD) Bangladesh: Norcuron | Norq | Survec | Vencur;
- (BE) Belgium: Norcuron;
- (BG) Bulgaria: Norcuron;
- (BR) Brazil: Norcuron | Vecuron | Veronio;
- (CH) Switzerland: Norcuron;
- (CI) Côte d'Ivoire: Norcuron;
- (CL) Chile: Bromuro de vecuronio;
- (CN) China: Norcuron | Wei Da Song | Xian lin;
- (CO) Colombia: Bromuro de vecuronio | Nodescron | Norcur | Norcuron | Vecuronio Bromuro;
- (CZ) Czech Republic: Norcuron;
- (DE) Germany: Norcuron | Vecuronium | Vecuronium bradex | Vecuronium Hikma;
- (DO) Dominican Republic: Vecuron;
- (EC) Ecuador: Norcuron | Vecuronio Bromuro;
- (EE) Estonia: Norcuron | Vecuronium inresa;
- (EG) Egypt: Norcuron;
- (FI) Finland: Norcuron;
- (FR) France: Norcuron;
- (GB) United Kingdom: Norcuron | Vecuronium sun;

- (GR) Greece: Norcuron;
- (GT) Guatemala: Vecuronio Bromuro;
- (HK) Hong Kong: Norcuron;
- (HR) Croatia: Norcuron;
- (HU) Hungary: Norcuron;
- (ID) Indonesia: Ecron | Norcuron;
- (IE) Ireland: Norcuron;
- (IL) Israel: Norcuron;
- (IN) India: Flexivec | Hetrovec | Kabivec | Krovec | Naprovec | Neovec | Norcuron | Survec | Troyvec | Vecubro | Vecuron | Veronate | Vibro;
- (IT) Italy: Norcuron;
- (JP) Japan: Musculax | Muscurate | Muscurate maruishi;
- (KE) Kenya: Neovec | Norcuron | Samvec | Vecuron;
- (KR) Korea, Republic of: Norcuron | Vecaron | Vecaronin | Vecron | Vecuron | Vecuronium | Veroni;
- (LB) Lebanon: Norcuron;
- (LT) Lithuania: Norcuron;
- (LV) Latvia: Norcuron;
- (MA) Morocco: Norcuron;
- (MX) Mexico: Bromivec | Curlem | Doxcurom | Nodescron | Norcuron | Vecuronio | Zellax;
- (MY) Malaysia: Norcuron;
- (NL) Netherlands: Norcuron;
- (NO) Norway: Norcuron | Vecuronium inresa | Vecuronium nordic prime;
- (NZ) New Zealand: Vecure | Vecuronium;
- (PE) Peru: Vecaron | Vecubrom | Vecuronio Bromuro;
- (PH) Philippines: Norcuron | Noruvec;
- (PK) Pakistan: Norcuron;
- (PL) Poland: Norcuron | Vecuronium inresa;
- (PR) Puerto Rico: Norcuron;
- (PT) Portugal: Norcuron;
- (PY) Paraguay: Nodescron | Vecural;
- (QA) Qatar: Norcuron;
- (RU) Russian Federation: Norcuron;
- (SA) Saudi Arabia: Norcuron;
- (SE) Sweden: Norcuron;
- (SG) Singapore: Norcuron;
- (SI) Slovenia: Norcuron | Veculex | Vecuronium inresa;
- (SK) Slovakia: Norcuron;
- (SR) Suriname: Norcuron;
- (SV) El Salvador: Nodescron;
- (TH) Thailand: Norcuron;
- (TR) Turkey: Blok I | Norcuron | Vecron | Vecubloc;
- (TW) Taiwan: Norcuron;
- (UA) Ukraine: Norcuron;
- (UY) Uruguay: Galaren | Vecural;
- (VE) Venezuela, Bolivarian Republic of: Norcuron | Prorelax | Vecuron | Vecuronio Bromuro;
- (ZA) South Africa: Norcuron

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